

Pregnancy schemes : **JSSK** launched 2011 to eliminate out-of-pocket expenses for institutional delivery and neonatal care. Covers all pregnant women delivering in public health institutions and sick newborns. Provides free drugs, diagnostics, blood (if needed), diet (3 days normal, 7 days C-section), and transport (home to facility, inter-facility, drop-back). In 2014 extended to cover all antenatal and postnatal pregnancy complications. **RKSK** targets adolescents (10–19 years), married and unmarried. Focus on reproductive and sexual health, nutrition, mental health, substance misuse, gender-based violence, NCDs. Delays marriage, reduces teen pregnancy, STIs, HIV. Delivers services via community outreach, school/family engagement, adolescent-friendly health clinics, routine checkups at all care levels. Emphasizes prevention and behavior change communication. **RBSK** targets children birth to 18 years. Detects and intervenes early for 4Ds: defects at birth, deficiencies, diseases, developmental delays/disabilities. Covers children in Anganwadi, govt, and aided schools. Uses mobile health teams. Ensures early treatment, reduces disease burden and family expenses.

Pregnancy hygiene and guidelines: Early registration within 12 weeks is essential. Conduct minimum four ANC check-ups (first within 12 weeks, others at 14–26, 28–34, 36 weeks to term). Administer 2 TT injections, provide 100 IFA tablets. Each ANC visit includes: history, physical exam (weight, BP, respiratory rate), check pallor, oedema, foetal growth, lie, FHS (after 24 weeks), lab tests (Hb, urine sugar/protein). Desirable: blood group (Rh), VDRL/RPR, HIV, HBsAg, blood sugar. Counselling covers birth preparedness, institutional delivery benefits, emergency planning (transport, funds, blood), signs of labour & complications, diet, rest, exclusive breastfeeding, safe sex, domestic violence risks, family planning, JSY/incentives info. Normal discomforts: nausea, vomiting, heartburn, constipation, frequent urination. Warning signs: fever, persistent vomiting, abnormal discharge, palpitations, breathlessness, general swelling, puffiness, headache, vision blur, low urine, burning micturition, bleeding, reduced fetal movement, watery discharge P/V. FHR normal: 120–160 bpm; outside this range → refer to MO. FHS not audible before 24 weeks. Initial Hb test forms baseline; compare with later readings; Sahli method usable at SC or outreach.

Pregnancy diet and rest: Pregnant women need ~300 kcal/day extra; postpartum ~500 kcal. High-risk groups needing more nutrition: underweight (<45kg), high physical activity, adolescents, pregnancies <2 years apart, multiple pregnancies, HIV+. Diet must include protein, iron, vitamin A, C, calcium, micronutrients. Key foods: cereals, dairy, leafy & other vegetables, pulses, eggs, meat, groundnuts, ragi, jaggery, fruits (mango, guava, orange, sweet lime, watermelon). Avoid tea/coffee/milk within 1 hour of meals to improve iron absorption. Take vitamin C-rich foods (e.g. lemon, amla) to aid iron uptake. High-fiber diet to prevent constipation. Dietary advice must consider local, seasonal, cultural, economic factors. Taboos against nutritious foods or for sex selection should be discouraged. PIH cases should not restrict salt/fluid intake. Advise 8 hours night sleep + 2 hours day rest. Avoid heavy work, lifting. Family should support. Strictly avoid alcohol, tobacco, addictive drugs. Avoid non-prescribed medication. Rest on left side, not flat on back (supine), to prevent supine hypotension syndrome; if needed, use a small pillow under pelvis.